

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°060/MVB_13/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (42)	• Haut-Uélé (1/13): Wamba • Ituri (26/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga and Ariwara. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Makiso-Kisangani, Lubunga and Mangobo
Reporting date	July 13, 2026
Publication date	July 14, 2026

KEY INDICATORS — CUMULATIVE AS OF JULY 13, 2026

 CONFIRMED CASES — 5 PROVINCES 2011 * +54 cases (Ituri 42 · N-Kivu 5 · H-Uélé 7)	 CONFIRMED DEATHS · LEATHITIS 754 · (37.5%)** +28 deaths (Ituri 18 · North Kivu 4 · Haut-Uélé 6)	 SUSPECTED CASES OF THE DAY 268 Ituri 174 · N-Kivu 62 · S-Kivu 4 · Haut-Uélé 28
 PATIENTS IN ISOLATION 753 Ituri 553 · North Kivu 170 · other 30	 HEALED — CUMULATIVE 366 +11 cured (Ituri) · Ituri 328 · N-Kivu 38	 CONTACT TRACKING — NATIONAL 67.4% 8,382 / 12,430 views · Ituri 62.0% · N-Kivu 92.0% · H-Uélé 55.5%

* Progress since 12/07 (1,957): +42 Ituri, +5 North Kivu and +7 Haut-Uélé (new net cases).

** Overall lethality on consolidated cumulative figures (754 / 2011); 34.9% in Ituri, 58.2% in North Kivu, 92.9% in Haut-Uélé.

RECONCILIATION NOTE — HAUT-UELE / NIA-NIA

Haut-Uélé has 14 confirmed cases: 1 initial case (Wamba, already counted on 12/07), 6 cases reclassified from Nia-Nia (Ituri) to Wamba, and 7 new cases (Wamba 1, Pawa 3, Boma Mangbetu 2, Isiro 1)

1. Epidemiological Highlights

ÿ **Ituri: 42 new confirmed cases** (Bunia: 16, Rwampara: 9, Nia-Nia: 6, Lita: 5, Nizi: 5, Lolwa: 1), 13 deaths of confirmed cases at CTE/CT and 5 community deaths.

ÿ **North Kivu: 5 new confirmed cases** (Katwa: 3, Beni: 1, Musienene: 1), **3 community deaths and 1 death** from Case confirmed at the CTE.

ÿ **Haut-Uélé: 7 new confirmed cases (Pawa: 3, Boma Mangbetu: 2, Isiro: 1, Wamba: 1) and 6 community deaths** distributed in the health zones of **Pawa: 3, Isiro: 1, Boma Mangbetu: 1, Wamba: 1**. It should be noted that the first seven (7) cases were imported from the health zone of Nia-Nia (Ituri), all of whom died in the community, and then seven (7) new cases were recorded on July 13, 2026.

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	1808	631	34.9%	26/36 (72.2%)	42
North Kivu	182	106	58.2%	11/34 (32.4%)	5
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Haut-Uélé	14	13	92.9%	4/13 (30.8%)	7
Tshopo	4	3	75.0%	3/23 (13.0%)	0
Total	2011	754	37.5%	45 out of 140	54

As of July 13, 2026, **Haut-Uélé** has 4 affected health zones (Wamba, Boma Mangbetu, Pawa and Isiro) after receiving laboratory results (INRB Kinshasa).

Provincial dynamics and case concentration

Ituri province accounts for **89.9% of cases (n=1,808)** and 83.7% of deaths (n=631), resulting in a crude case fatality rate of 34.9%, remaining the epicenter of the epidemic. The health zones of **Bunia (534 cases), Rwampara (405), Mongbwalu (336), Nizi (131), and Nyankunde (99)** remain the most affected. New confirmed cases recorded as of July 13th are distributed across the health zones of Bunia (16), Rwampara (9), Nia-Nia (6), Lita (5), Nizi (5), Katwa (3), Pawa (3), Boma Mangbetu (2), Beni (1), Isiro (1), Lolwa (1), Musienene (1), and Wamba (1).

Severity of the epidemic in North Kivu

As of July 13, North Kivu had recorded **182 confirmed cases and 106 deaths**, representing a **crude case fatality rate of 58.2 %**. The highest number of reported cases was in Katwa (n=76), Butembo (n=46), and Beni (n=31). The particularly high case fatality rate in North Kivu should be interpreted with caution in the context of an evolving epidemic, but remains consistent with late detection of cases and underreporting of less severe forms.

Geographic extension — Haut-Uélé and Tshopo

Haut -Uélé province has reported **14 confirmed cases** (8 in Wamba, 3 in Pawa, 2 in Boma Mangbetu, and 1 in Isiro) **and 13 deaths**. However, the first 7 cases, including the province's index case, have been reclassified as imported cases from the Nia-Nia health zone (Ituri), and 7 new cases were recorded as of July 13, 2026. Thus, 4 health zones are affected in the Haut-Uélé health district, but Aba and Doruma remain on alert. Meanwhile, **Tshopo** province remains at 3 affected zones (Makiso-Kisangani, Mangobo, and Lubunga; 4 cases, 3 deaths). All these cases are linked to the epicenter in Ituri (Nia-Nia/PK51). Proximity to Uganda via Aru/Ariwara increases the risk of further spread.

2.2 — Distribution by health zone as of 13/07/2026

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community death res (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	1808	631	34.9% 42		5	13	18
Bunia	534	172	32.2%	16	3	3	6
Rwampara	405	95	23.5%	9	0	5	5
Mongbwalu	336	182	54.2%			1	1
Nizi	131	50	38.2%	5	2	0	2
Nyankunde	99	26	26.3%			1	1
Mangala	50	29	58.0%				
Lita	59	23	39.0%	5	0	1	1
Nia-Nia	36	7	19.4%	6	0	0	0
Bamboo	31	10	32.3%				
Komanda	27	10	37.0%	0	0	2	2
Tchomia	19	4	21.1%				
Unidentified	17	0	0.0%				
Kilo	9	1	11.1%				
Mandima	8	3	37.5%				
Logo	7	0	0.0%				
Aungba	6	2	33.3%				
Damascus	5	0	0.0%				
Mambasa	5	4	80.0%				

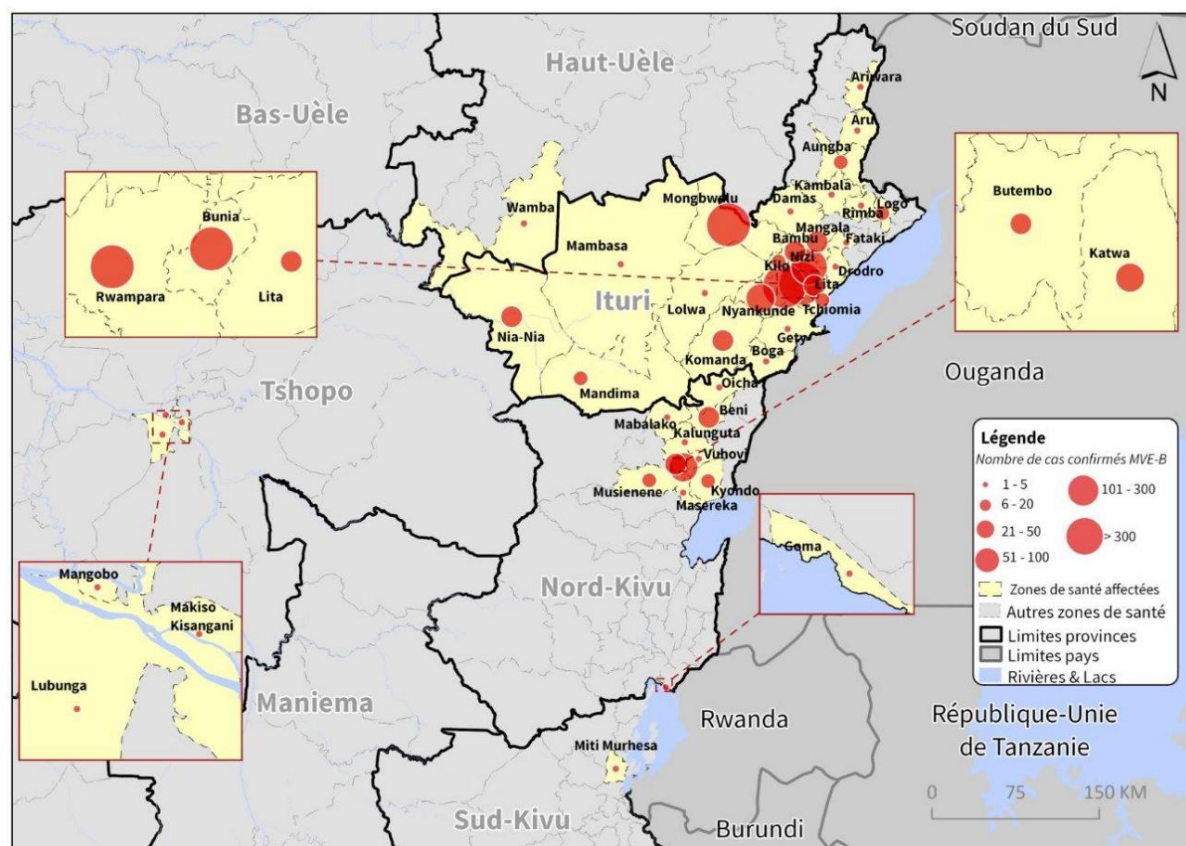
Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community death res (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Rimba	4	0	0.0%				
Fataki	5	4	80.0%				
Drodro	5	3	60.0%				
Aru	3	1	33.3%				
Kambala	2	2	100.0%				
Gety	1	0	0.0%				
Lolwa	2	1	50.0%	1	0	0	0
Boga	1	1	100.0%				
Ariwara	1	1	100.0%				
North Kivu	182	106	58.2%	5	3	1	4
Katwa	76	55	72.4%	3	3	1	4
Butembo	46	22	47.8%				
Blessed	31	20	64.5%	1			
Musienene	11	2	18.2%	1			
Kyondo	7	3	42.9%				
Kalunguta	2	1	50.0%				
Masereka	3	0	0.0%				
Oicha	3	2	66.7%				
Goma	1	0	0.0%				
Vuhovi	1	1	100.0%				
Mabalako	1	0	0.0%				
South Kivu	3	1	33.3%	47	8	14	22
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	14	13	92.9%	7	6		6
Wamba	8	8	100.0%	1	1		1
Pawa	3	3	100.0%	3	3		3
Boma Mangbetu	2	1	50.0%	2	1		1
Isiro	1	1	100.0%	1	1		1
Tshopo	4	3	75.0%	—	—		
Makiso-Kisangani	2	2	100.0%				
Mangobo	1	0	0.0%				
Lubunga	1	1	100.0%				
Total	2011	754	37.5%	54	14	14	28

The persistent concentration of cases in a limited number of health zones suggests that the current dynamics are primarily driven by a few major transmission hotspots. Targeted intensification of investigations, contact tracing, and community interventions in these areas should have a greater impact than a uniform dispersal of resources across all affected areas.

2.3 — Mapping of confirmed cases

The spatial distribution confirms the concentration of cases in the health zones of — Bunia, Rwampara, Mongbwalu, Nizi, Nyankunde — with an extension towards North Kivu (Beni, Butembo, Katwa) and Haut-Uélé.

FIGURE 1 — HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES (AS OF 12/07/2026)



Transmission remains highly concentrated in Ituri, which continues to account for the bulk of the national epidemic burden. However, the recent emergence of new outbreaks in Haut-Uélé confirms a progressive expansion of the affected area, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 — Temporal analysis — epidemic curves

We are seeing an increasing number of confirmed cases from week to week, reflecting ongoing community transmission. Epidemiological weeks S25 and S26 confirm this continued growth, with more than 300 confirmed cases per week.

FIGURE 2 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

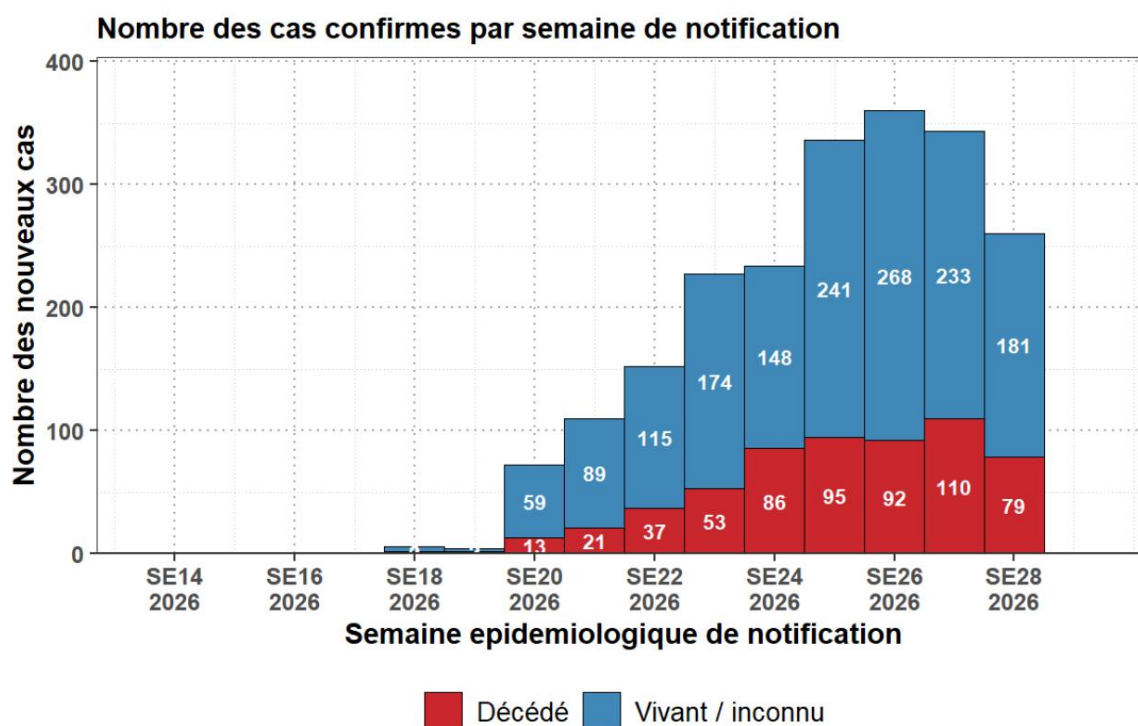
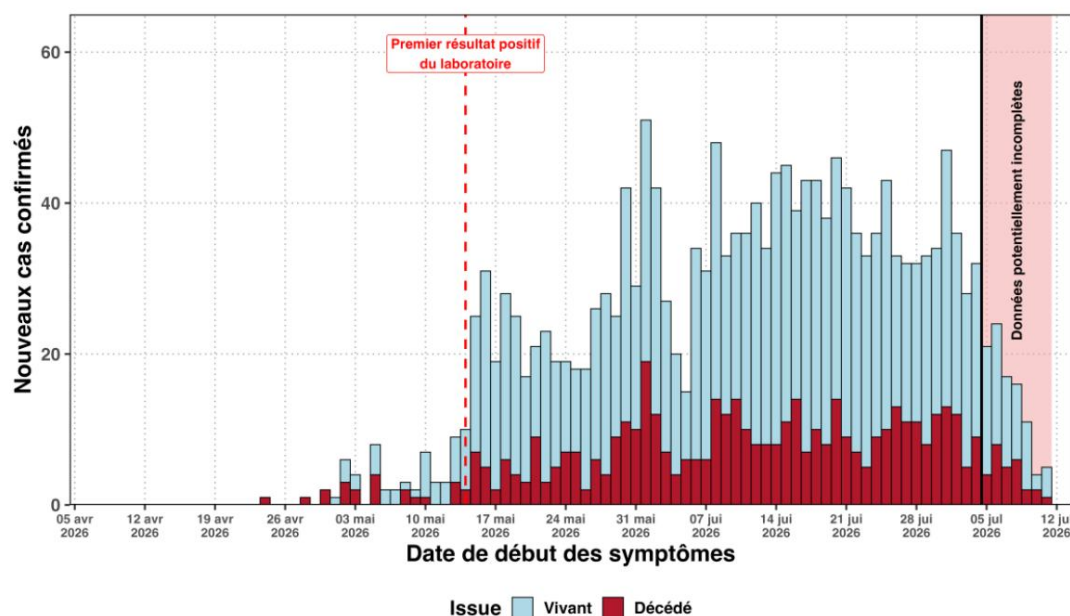


FIGURE 3 — EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)



The epidemic continues a phase of sustained transmission, characterized by a rapid increase in the number of cases since the outbreak began, followed by a period of relative stabilization at a still high level of transmission. Fluctuations observed in recent days should be interpreted with caution due to reporting delays and the ongoing consolidation of data.

2.5 — Demographic characteristics

The age pyramid shows a predominance of **working-age adults** (18–50 years). Minors under 18 and people over 50 are proportionally less affected.

FIGURE 4 — CONFIRMED CASES BY SEX AND AGE GROUP

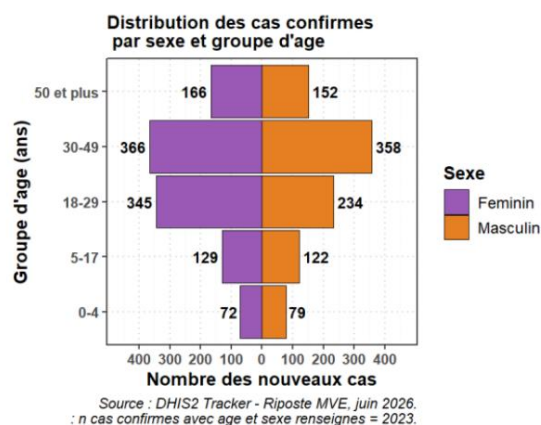
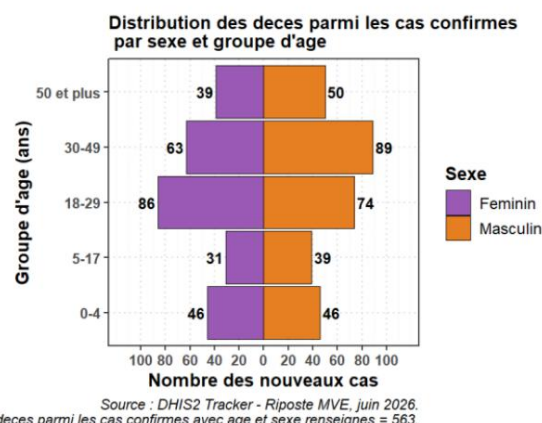


FIGURE 5 — DEATHS BY SEX AND AGE GROUP (AMONG THE (CONFIRMED CASES))



The distribution by sex remains generally balanced, with no major differences suggesting preferential transmission by sex. The observed differences are more likely related to exposure patterns than to different biological susceptibility. The concentration of cases among working-age adults is consistent with more frequent community and occupational exposures. However, the significant proportion of pediatric cases confirms the persistence of intrafamilial transmission in several affected areas.

3 Monitoring, testing and reporting

3.1 — Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS (NORTH KIVU AS OF 13/07; OTHERS POSTPONED FROM 12/07)

Indicator	Ituri	North Kivu	South Kivu	Haut-Uélé	Total
Report (D-1)	17	29	0	0	46
New alerts received	408	665	5	32	1110
Total alerts for the day	425	694	5	32	1,156
Alerts investigated	348	665	5	31	1049
Investigation rate (24 h)	81.9%	95.8% 100.0%		96.9%	90.5%
Alerts confirmed — live	118	23	4	11	156
Alerts confirmed — deceased (comm.)	56	39	0	17	112
Total suspected cases for today	174	62	4	28	268

The high volume of alerts investigated reflects the continuation of active surveillance, although provincial disparities persist in the speed of investigations.

3.2 — Laboratory: tests and positivity

📍 **Ituri:** 229 samples analyzed (208 blood, 21 swab) of which **42 were positive**, representing a positivity rate of 18.3%

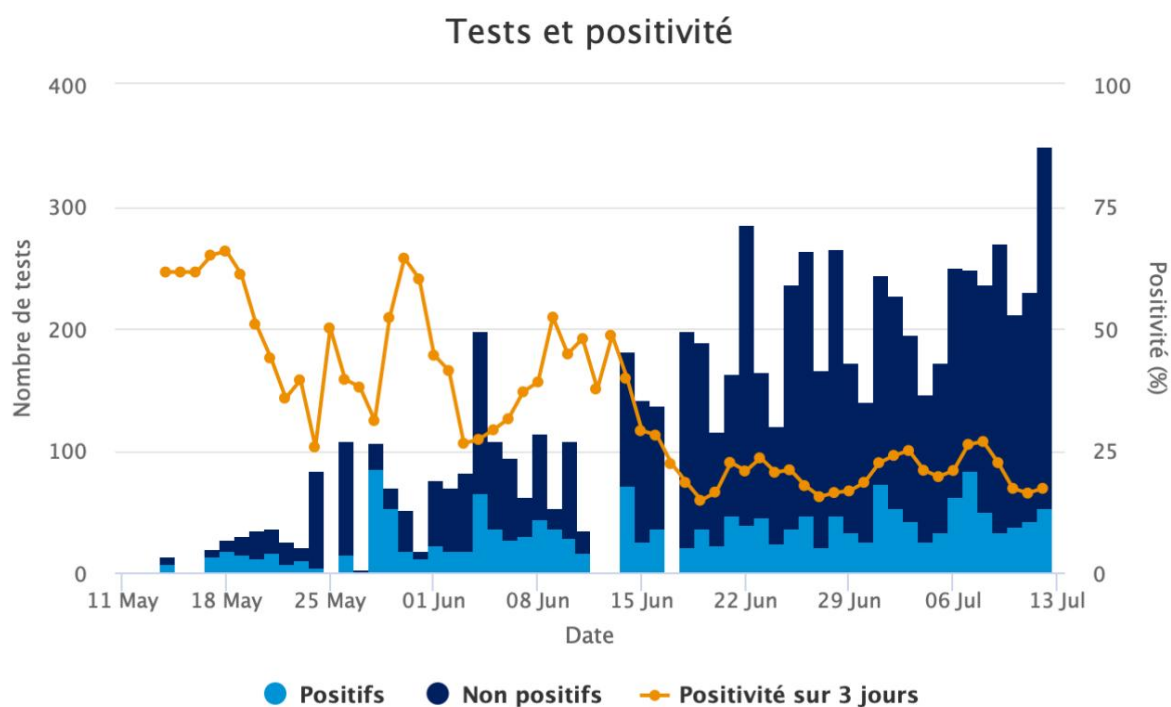
📍 **North Kivu:** 99 samples analyzed, of which **5 were positive** (positivity rate 6.5%),

📍 **South Kivu:** 4 samples collected, currently being analyzed

Diagnostic capacity remains under considerable strain. On July 13, Ituri analyzed **229 samples, with 42 positive results** (positivity rate approximately 18%); plans are underway to establish local laboratories in Mambasa and Logo. North Kivu continues to have a low testing rate (6.5%), and the backlog of samples remains to be cleared.

The observed positivity confirms the continuation of active transmission, particularly in Ituri, while maintaining diagnostic delays remains a critical factor for the rapid interruption of transmission chains.

FIGURE 6 — TESTS AND POSITIVITY (COUSP SITUATION REPORT)



3.3 — Contact Tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	10,011	6,206	62.0%
North Kivu	2,279	2,108	92.0%
Haut-Uélé	140	68	55.5%
South Kivu (end of monitoring)	0	0	n / A
Total	12,430	8,382	67.4%

Ituri (July 13): 6,206 contacts seen out of 10,011 (62.0%), 509 new contacts listed, and 21 identified as suspects; seven health zones did not submit their follow-up reports (Ariwara, Lolwa, Logo, Rimba, Kambala, Damas, Gethy). North Kivu (July 13): 2,108 contacts seen out of 2,279 (92%), 62 new contacts listed, and 14 identified as suspects. In Haut-Uélé, 68 contacts followed up out of 140 (55.5%, below target): 85 in Wamba, 32 in Isiro, and 23 in Pawa. The overall rate (67.4%) remains below the recommended operational threshold (95%).

3.4 Entry points and control points (PoE/PoC)

The **71 checkpoints (PoCs) and points of entry (PoEs)** remain activated and reinforced (Ituri 57, North Kivu 14). As of July 13, **154,481 travelers** had been screened (Ituri 94,294, North Kivu 60,187), with a screening rate of approximately 96% and an awareness rate of 98%. Seven (7) alerts were notified: 6 in Ituri (5 confirmed cases of living individuals referred to the CT) and 1 in North Kivu (1 body intercepted).

3.4.1 — Crossing point alerts (24 h)

Ituri — 6 alerts (5 confirmed active)

At **PoC Ombayi** (2 alerts, one confirmed): a 17-year-old girl (Otsokotsu market y Kaliko, fever/headache) confirmed, isolated and transferred to the CT. At **PoC Makoko 2** (4 alerts): 4 suspected cases (including PK3 Arua 2, PK44yMambasa, PK4 Mumbere road) confirmed, isolated and transferred to the transit center.

North Kivu — 1 alert (invalidated)

A live alert was reported and subsequently invalidated after thorough investigation at the **Eringeti PoC**. A field visit was conducted with the Oicha MCZ (Military Coordination Zone) with a view to activating and integrating the Matombo PoC (Oicha–Eringeti axis).

3.4.2 — Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC), AS OF 13/07/2026

Indicators	Ituri	North Kivu	Overall
Completeness of daily reports	93% (53/57)	100% (14/14)	94% (67/71)
Proportion of PoC enabled and PoE enhanced	100% (57/57)	100% (14/14)	100% (71/71)
Number of people who have switched to PoE/PoC	94,294	60,187	154,481
% of travelers screened	94%	98%	96%
% of travelers who washed their hands	93%	98%	95%
% of travelers made aware	98.3%	98%	98.2%
Number of alerts notified	6	1	7
Number of validated live alerts	5	0	5
Number of bodies recovered today	0	1	1
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100%	so	100%
% of the lifeless bodies swabed	so	100%	100%
Number of positive results of referred suspected cases	0	0	0

Indicators for the PoE/PoC pillar as of July 13, 2026 (Ituri + North Kivu). Refusal of testing: 6% in Ituri, 2% in North Kivu. No laboratory feedback on referred suspected cases was recorded today.

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI), AS OF 13/07/2026

Indicator	Ituri	North Kivu	South Kivu	Haut-Uélé	Together
Patients in bed (Day -1)	538	173	30	1	742
Total admissions (24 hours)	85	12	4	2	103
Exits	15	2	1	0	18
deceased					

Indicator	Ituri	North Kivu	South Kivu	Haut-Uélé	Together
No case	33	11	6	0	50
Healed	13	2	0	0	15
Escapees	9	0	0	0	9
Transferred to the HGR	0	0	0	0	0
Total outflows (24 h)	70	15	7	0	92
Patients in isolation (end of day)	553	170	27	3	753
including those confirmed	229	17	0	0	246
including suspects	324	153	27	3	507
Occupancy rate	82.7%	120.6%	60.0%	100%	87.4%

North Kivu remains at capacity (120.6%), with a critical shortage of isolation beds; Ituri (82.7%) remains under pressure. A total of 753 patients are in isolation (246 confirmed, 507 suspected), with an overall occupancy rate of 87.4%.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY INDICATORS SMSPS, JULY 13, 2026

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	10 (27.8%)	0
Confirmed cases are being monitored.	67 (53.2%)	8 (100%)
New suspected cases	5 (15.6%)	15 (100%)
Suspected cases are being monitored	80 (69.0%)	15 (100%)
Laboratory results announced (including positive ones)	19 (10)	6 (0)
Children separated — nursery / community	5/0	0 / 0
CTE support staff	19	1
Frontline Personnel (PPL)	1	0
Household/Community Members	23	0
EDS supported (SMSPS)	2	0

Key SMSPS highlight of July 13: Start of training for service providers (clinical psychologists, APS, TPS, RECO — 27 women, 23 men) in the Nyankunde Health Zone. Source: Key point SMSPS Ituri & North Kivu, July 13, 2026.

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD (CUMULATIVE AS OF JULY 13, 2026, ITURI)

Health Zone	Confirmed cases	Healed	In hospital	Death
Bunia	35	16	5	14
Rwampara	32	23	2	7
Mongbwalu	16	5	3	8
Nyankunde	14	11	1	2
Nia-Nia	5	0	5	0
Nizi	3	0	0	3

Health Zone	Confirmed cases	Healed	In hospital	Death
Mangala	3	1	1	1
Others (Kilo, Bambu, Lita)	6	2	3	1
Total — case fatality rate 31.6%	114	58	20	36

PPL = healthcare workers and frontline providers. Source: Continuity of Essential Services pillar (CSSE), July 13, 2026. The high infection rate among staff (case fatality rate 31.6%) underscores the need for IPC and PPE.

7. Response activities by pillar — summary as of July 13, 2026

COORDINATION ȳ Coordination meetings (PTF) and field supervision. Development of the 3-month roadmap for Haut-Uélé and joint DPS/WHO/UNICEF mission to the treatment site in Isiro (HGR Isiro). Working visit of the Minister to Kisangani; advocacy for ambulances and payment of staff. ȳ Finalization of the permanent construction plan for a 100-bed CTE by the INSP Finalization of the construction plan for the Ebola Survivors Clinic (MVE-17) ȳ Holding of the technical meeting with the provinces of Ituri, North Kivu, South Kivu, Haut Uele and Tshopo as part of the extension. Continued recruitment of local staff in collaboration with the medical association, the nursing association, and the provincial health division	PCI / WASH ȳ ESS of confirmed cases decontaminated (99%); 924 service providers trained. North Kivu: morgue decontamination and ESS (Beni, Katwa, Kalunguta); 4 infected PPLs in Tchomia (2 deaths); training PCI pending.
Dignified and secure burials (DSM) ȳ North Kivu: 28 death alerts, 25 bodies swabed and secured, 3 not swabed (refusal by families in Butembo), 29 EDS carried out. Ituri: paralysis of activities in Rwampara (non-payment of EDS teams), 9 reports from Nizi in progress, resistance in Mandima; planned allocation of 20 EDS kits to Nizi.	HOLISTIC CARE (HCC) ȳ Ituri: 11 new recoveries (including a 6-month-old child); 14 Deaths among confirmed cases were recorded in the Ebola Treatment Centers (ETCs) of Ituri: 3 at the CTC CME, 2 at the Bunia ETC, 2 at the ISTE ETC, 2 at the Komanda ETC, 1 at the Elikya ETC, 1 at the Nyankunde ETC, 1 at the Mongbwalu ETC. ȳ Ituri: food support to 1,084 beneficiaries — hot meals to suspected cases (119), confirmed cases (174), people with HIV (43), and households (748); 19 specific programs (daycare, caregivers). South Kivu: hot rations to cases in isolation.
COMMUNICATION (RCCE / CREC) ȳ Ituri: 5 patients discharged from the Nizi CTE (negative results — first wave); 2,958 people reached through home visits, 3,664 through talks, 3,755 through mass awareness campaigns; 36 radio stations involved. Feedback: 58 collected, 11 clarified.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ȳ 20/36 ZS of Ituri shared the mapping of partners offering free care; Cap MEG follow-up; follow-up of frontline staff assigned (see Table 8).
RESEARCH, INNOVATION & VACCINATION ȳ Analysis and submission of the CAP protocol (Ituri communities, CRS support); anthropological research in Mongbwalu; official launch of the EBO-PEP clinical trials (ALIMA) and continuation of the vaccine trial (Partners); INRB / ALIMA / WHO meeting.	LOGISTICS ȳ Ituri: provision of a Hadjin motorcycle to the PoE/PoC pillar; installation of local laboratories planned in Mambasa and Logo. North Kivu: deployment of teams; delivery of IPC kits and medicines; repair of mobile vehicle 103 (ICCN, Mutsora).
SECURITY ȳ No new major incidents reported on July 13; securing PoE/PoC providers (PoC Matombo, ZS Oicha); briefing of FARD, PNC, DGM and ANR teams at PoC PK5; support for EDS teams.	

PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH)

Ituri: mass awareness campaigns continued (focus on people living with disabilities); 368 people sensitized today (cumulative 13,781); cumulative total of 2,295 people briefed and signed the codes of good conduct in 5 health zones (Bunia, Rwampara, Mongwalu, Lita, Aru); revitalization of the 12 CBCMs in Mongwalu.

In summary, response activities continue across all pillars, but several operational constraints continue to limit the speed and effectiveness of interventions in the main transmission hotspots.

8 Main challenges, impacts and actions required

Challenge No.		Impact	Actions required
1	Reluctance to undergo post-mortem sampling (EDS/swabs) and resistance community	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of EDS teams (paralysis in Rwampara)	Disruption of dignified and safe burials	Regularize payments to EDS service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT facilities and saturation in North Kivu (120.6%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures
4	Contact tracing performance not optimal (Haut-Uélé 55.5%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs); enhanced supervision
5	Ambulance shortages and pre-positioning at points of control	Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (114 cases of PPL including 36 deaths)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Isiro, Mambasa, Logo)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Insufficient financial resources	Limitation of the implementation of the pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Community briefing in ZS MONGBWALU



Awareness-raising in ZS LITA

**POUR TOUTE INFORMATION
SUPPLÉMENTAIRE, VEUILLEZ CONTACTER**

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**Avec l'appui technique de l'Organisation Mondiale
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